



EVERGREEN COMMUNITY CARE INC.

ORGANIZATIONAL DIVISIONS

All Important Areas for a Residential Care Agency
Aligned to MCFD Standards for Staffed Children's Residential Services (SHSS) — April 2026

Document Reference	ECC-DIV-2026-001
Version	1.0 — July 2026
Prepared By	Evergreen Community Care Inc.
Approved By	Executive Director
Effective Date	July 1, 2026
Review Date	July 1, 2027
Purpose	Defines all organizational divisions and key operational areas for Evergreen Community Care's residential care programs. Establishes clear ownership, governing standards, and document references for every function.
Divisions	1. Governance and Leadership 2. Program Operations 3. Human Resources 4. Finance and Administration 5. Quality Assurance 6. OHS and Workplace Safety 7. External Relations
Related Documents	RC Program Manual (ECC-RC-POL-2026-003); HR Policy Binder (ECC-HR-POL-2026-001); OHS Program Binder (ECC-OHS-POL-2026-002); QA Program Binder (ECC-QA-2026-001); Finance and Administration Manual (ECC-FA-2026-001); PC Manual (ECC-PC-MAN-2026-001); PM Manual (ECC-PM-MAN-2026-001)



INTRODUCTION

Evergreen Community Care Inc. operates licensed residential care programs for children and youth placed by the Ministry of Children and Family Development (MCFD). This document defines the seven organizational divisions that govern every aspect of the agency's operation — from front-line care delivery to Board governance, from financial management to external regulatory relationships.

Each division is described with its purpose, key areas, responsible roles, governing MCFD Standards and legislation, and the internal documents that implement its requirements. Together, these seven divisions form the complete organizational architecture of Evergreen Community Care.

How the Divisions Interlock

No division operates independently. The seven divisions are interdependent in specific, predictable ways:

When this event occurs...	These divisions are triggered...	Primary governing document
New youth admitted to the program	Program Operations (admission); HR (staffing); Finance (per-diem); QA (ISP compliance)	RC Program Manual s.16; PC Manual App. B
Category 1 Critical Incident occurs	Program Operations (response); OHS (if worker injury); QA (post-CI review); External (MCFD notification)	RC Program Manual s.26; PM Manual s.4
New staff member hired	HR (CRC, screening, file); Program Operations (orientation); OHS (safety orientation); Finance (payroll)	HR Policy Binder s.3; PC Manual App. B
Mandatory training lapses for a staff member	HR (compliance, discipline); Program Operations (duty removal); QA (KPI impact)	PC Manual App. E; HR Policy Binder s.5
Licensing inspection occurs	Governance (ED notification); Program Operations (facilitation); QA (self-assessment); External (Licensing Officer)	PM Manual s.2.3; QA Binder Tool A
Indigenous youth admitted	Program Operations (Cultural Plan within 30 days); External (Nation/DAA notification); QA (Cultural Plan tracking)	RC Program Manual s.31; PC Manual s.6
Annual review cycle (July)	All seven divisions participate in the Annual Program Review (MCFD Standard H.2)	QA Program Binder s.5; PM Manual s.12.5



DIVISION 1 — GOVERNANCE AND LEADERSHIP

Board of Directors · Executive Director · Organizational Accountability

The Governance and Leadership division is responsible for the ultimate accountability of Evergreen Community Care. The Board provides governance; the Executive Director provides operational leadership. No policy, contract, or organizational commitment is binding without authorization from this division.

1.1 Board of Directors

Purpose: Holds governance authority over the organization. Approves policy, budget, and organizational direction. Accountable to the public, to MCFD, and to the communities Evergreen Community Care serves.

Key Responsibilities: Approve the annual budget and organizational policy; receive the annual program review and QA summary; hold the Executive Director accountable for safety and quality outcomes; ensure adequate resources for OHS compliance; maintain governance accountability for all MCFD contractual obligations

MCFD Alignment: Standard H.1 (Organization and Management); Standard H.2 (Review and Evaluation) — Board receives annual program review

OHS Role: Workers Compensation Act s.115 — Board approves the OHS Program, annual OHS budget, and receives the annual OHS performance report

Meeting Requirements: Minimum quarterly Board meetings; annual program review is a standing Board agenda item; emergency meetings within 48 hours of any serious incident with organizational liability implications

 **Governing documents:** *RC Program Manual v4 (ECC-RC-POL-2026-003); All Policy Binders; Finance and Administration Manual*

1.2 Executive Director

Purpose: Holds ultimate operational accountability for Evergreen Community Care. The ED is the organizational authority for all decisions with MCFD contract, licensing, legal, media, or financial liability implications.

Authority: Only the ED (or delegated Board authority) may: sign employment offers and contracts; commit the organization to service agreements; make public statements to media; authorize capital expenditures above the PM delegated limit; settle any legal claim; terminate employment

MCFD Relationship: Organizational signatory on all MCFD contracts; accountable to MCFD Contract Manager and Regional Director; receives and acts on all licensing orders and WorkSafeBC orders

Supervision of PM: ED provides monthly reflective supervision to Program Managers; conducts PM annual performance evaluations; approves PM PIPs; signs off on PM progressive discipline

Board Relationship: Reports to the Board; presents quarterly KPI dashboard; leads the annual program review; escalates all matters with board-level risk implications

 **Governing documents:** *Finance Manual (ECC-FA-2026-001); PM Manual (ECC-PM-MAN-2026-001); HR Policy Binder (ECC-HR-POL-2026-001)*

1.3 Strategic Planning and Organizational Direction



Purpose: Ensures Evergreen Community Care has a defined direction, clear priorities, and the organizational capacity to deliver on its MCFD contracted commitments and its mission to children and youth in care.

Annual Planning Cycle: Annual organizational priorities set by ED and Board each September; budget developed concurrently (September–November); service model reviewed annually in July alongside the Annual Program Review

Reconciliation Commitment: Organizational reconciliation commitments reviewed annually; progress against TRC Calls to Action 65 (Indigenous child welfare) documented; DRIPA alignment assessed in annual program review

Policy Governance: All policy manuals reviewed annually in July; changes communicated to all affected staff within 30 days with signed acknowledgement; MCFD and Licensing Officer notified of material policy changes

Policy Document	Review Cycle
RC Program Manual (ECC-RC-POL-2026-003)	Annual — July; also upon MCFD Standards amendment or licensing recommendation
HR Policy Binder (ECC-HR-POL-2026-001)	Annual — July; also upon amendment to BC ESA, Human Rights Code, or Labour Relations Code
OHS Program Binder (ECC-OHS-POL-2026-002)	Annual — July; also upon serious incident, WCA amendment, or WorkSafeBC order
QA Program Binder (ECC-QA-2026-001)	Annual — July; alongside Annual Program Review
Finance and Administration Manual (ECC-FA-2026-001)	Annual — upon organizational budget cycle; also upon MCFD financial reporting change
PC Manual (ECC-PC-MAN-2026-001)	Annual — July; upon any change to MCFD Standards or organizational structure
PM Manual (ECC-PM-MAN-2026-001)	Annual — July; upon any change to MCFD Standards or organizational structure
Organizational Divisions (ECC-DIV-2026-001)	Annual — July

 **Governing documents:** *RC Program Manual v4; Finance Manual; All Policy Binders*



DIVISION 2 — PROGRAM OPERATIONS

MCFD Standards A–F · Program Manager · Program Coordinator · Youth Care Workers

The Program Operations division delivers the core residential care service to children and youth placed by MCFD. It encompasses every aspect of the day-to-day program — from admission to discharge, from daily care to crisis response, from ISP development to cultural connection. This division is directly governed by MCFD Standards A through F.

2.1 Standard A — Program Purpose and Description

MCFD Standard: A.1 — Written Program Description required

Key Requirements: Current written Program Description maintained and distributed to: each youth at admission; each new staff member at orientation; MCFD and Licensing Officer on request; public on request

Responsible Role: Program Manager (owns and maintains); Program Coordinator (distributes at admission and orientation)

Annual Action: Review and update in July; notify MCFD of any material program change before implementation; all staff sign updated Program Description within 30 days

Licensing: Program must operate within all licence conditions at all times — capacity, population served, staffing model; Licensing Officer notified of any parameter change before implementation

 **Governing documents:** *RC Program Manual v4 s.3; PC Manual s.2; PM Manual s.2*

2.2 Standard B — Rights, Complaints, and Privacy

MCFD Standard: B.1–B.5 — Rights of children and youth in care

CFCSA s.70 Rights: Every youth in care has 14 legal rights including: freedom from corporal punishment; reasonable privacy; family contact; access to medical and dental care; cultural heritage and language; access to the Advocate (1-800-476-3933), RCY, and Ombudsperson — without restriction, without permission, at any time

Complaint Resolution: Five-step process: YCW/staff acknowledgement (same shift) → PC investigation (5 business days) → PC written response (10 business days) → PM-level investigation (escalation) → ED review (final internal level)

Absolute Rule: No staff member at any level may interfere with a youth's access to the Advocate, Ombudsperson, RCY, MCFD social worker, or police. Any interference is a Category 1 Critical Incident and grounds for immediate suspension

Privacy / FOIPPA: Every youth's personal information is confidential; only shared with those who need it for care purposes; any privacy breach contained and reported to PM within 24 hours; media inquiries redirected to ED only

 **Governing documents:** *RC Program Manual v4 s.4; PC Manual s.3; PM Manual s.3*

2.3 Standard C — Personal Safety

MCFD Standard: C.1–C.9 — Personal safety of children and youth

Environmental Safety: Monthly facility safety inspection; hot water at all taps maximum 49°C (RCR April 2026 amendment); medication cabinet locked; chemical storage locked; environmental risk assessment at admission and annually

Critical Incident Categories: Category 1 (same day — verbal within 2 hours, written within 24 hours): death, hospitalization, abuse allegation, suicidal crisis, self-harm with injury, missing youth,



police contact, physical restraint, school suspension | Category 2 (next business day): medication error, property damage >\$500, unauthorized overnight | Category 3 (weekly report): pattern concerns

MCFD After-Hours: Provincial Centralized Screening (24/7): 1-800-663-9122 — use when social worker unavailable for any Category 1 incident

Physical Restraint: Last resort only; imminent serious physical harm threshold; current NVCI certification required; every use is a Category 1 CI; prone restraint and seclusion are absolutely prohibited under all circumstances

Missing Youth Protocol: Phase 1 (prudent parent, maximum 30 minutes): phone, search, known contacts | Phase 2 (confirmed missing): MCFD PCS, 911, PM, ED notification; chronological log; welfare check and ISP review on return

Self-Harm Protocol: Stay present; remove means; calm engagement; ask directly (reduces risk); supervisor immediately; 911 if active danger; MCFD within 2 hours; Crisis Centre BC 1-800-784-2433; Kids Help Phone 1-800-668-6868

 **Governing documents:** *RC Program Manual v4 s.5, s.26–30; PC Manual s.4; PM Manual s.4*

2.4 Standard D — Quality of Service Experiences

MCFD Standard: D.1–D.8 — Quality of service experiences

Daily Environment: The program creates a home — not an institution. Three nutritious meals and two snacks daily; personal belongings secured; privacy respected; schedule that reflects youth's preferences; genuine therapeutic relationships

Gender-Affirming Care: MANDATORY — Standard D.1.13(b), D.6(g), G.3.2(b), G.4.3(c) all amended April 2024. Every staff member uses correct name and pronouns for every youth consistently from Day 1. Deliberate misgendering is a Code of Ethics violation subject to disciplinary action. Gender identity is confidential without informed consent.

Family Involvement: Family contact is a legal right (CFCSA s.70), not a privilege. Family Contact Plan in every ISP. Contact never withheld as a consequence. For Indigenous youth: Nation, DAA, Elder, and cultural community contact actively facilitated.

Behaviour Support: Trauma-informed, attachment-based positive behaviour support. Behaviour Support Plan (BSP) individualized for each youth. All prohibited practices absolutely forbidden: corporal punishment, seclusion, withholding meals, humiliation, unauthorized family restriction, threatening removal.

Water Temperature: Residential Care Regulation April 2026 amendment: all beverages, bottles, and heated food provided to youth checked for safe temperature before serving; bathing water maximum 42°C; recommended 37–38°C

Youth Voice: Youth input genuinely shapes care plans, daily routines, programming, and organizational learning. Not token signatures on documents — documented consultation at every ISP review.

 **Governing documents:** *RC Program Manual v4 s.6, s.21–25, s.32–34; PC Manual s.5; PM Manual s.5*

2.5 Standard E — Child and Youth Service Planning

MCFD Standard: E.1–E.7 — Child and youth service planning

ISP Requirements: Preliminary ISP within 48 hours of admission; full ISP meeting within 14 days; quarterly review minimum; emergency review within 5 business days of any Category 1 CI. ISP approved by Program Manager before MCFD submission. All 14 required ISP components including: strengths, goals by domain, BSP, safety plan, medication plan, cultural plan, gender affirmation plan, youth signature/acknowledgement.



Cultural Plan: Every Indigenous child or youth has a Cultural Plan developed within 30 days of admission, in genuine collaboration with the youth, their family, and their Nation. Updated at every ISP review. Documents: Nation and community affiliation, Elders and cultural mentors, specific activities and ceremonies, traditional foods and medicines, language learning, DAA involvement.

Transition Planning: Begins 60 days before anticipated discharge. Youth meaningfully involved. Full transition plan: receiving provider documentation, school transfer, new physician, medication supply, personal ID, cultural connections confirmed, community supports. Warm handover. Discharge summary within 5 business days.

School: Every school-aged youth enrolled within 5 school days of admission. Attendance tracked daily. School suspensions reported as Category 1 CI on same day. PM or PC attends all IEP reviews and case conferences.

Health Care: Every youth established with GP or NP within 30 days. Annual dental exam. Emergency: 911 immediately, staff accompanies to hospital, MCFD within 1 hour. Mental health referral within 30 days of identification.

 **Governing documents:** *RC Program Manual v4 s.7, s.16–20; PC Manual s.6; PM Manual s.6*

2.6 Standard F — Shelter and Equipment

MCFD Standard: F.1–F.4 — Shelter and equipment

Facility Compliance: Program operates within all licence conditions; current licence posted prominently; all BC Building Code, BC Fire Code, and RCR physical standards maintained

Monthly Inspection: Program Coordinator completes Monthly Facility Safety Inspection (Appendix C of PC Manual) including: fire safety, physical environment, health and safety equipment, medication and records. All deficiencies assigned priority (1=immediate, 2=48 hours, 3=14 days), owner, and target date. Program Manager reviews and signs off within 5 business days.

Fire Safety: Smoke and CO detectors tested monthly; fire extinguishers visually checked monthly; emergency lighting tested monthly; evacuation plan posted every floor; quarterly announced fire drill; annual unannounced night/weekend drill. All drills documented.

Hot Water: Maximum 49°C at all taps (RCR April 2026); thermostatic mixing valves installed where required; temperature confirmed monthly and at every admission

Vehicles: All program vehicles maintained in good running order; current insurance and registration; pre-trip inspection log; monthly formal check; driver's abstract on file for all drivers, reviewed annually

 **Governing documents:** *RC Program Manual v4 s.8, s.30; PC Manual s.7; PM Manual s.7*



DIVISION 3 — HUMAN RESOURCES

MCFD Standard G · HR Policy Binder · ESA · BC Human Rights Code

The Human Resources division governs the complete employment lifecycle at Evergreen Community Care — from recruitment and screening through to separation. It is directly governed by MCFD Standard G (G.1 through G.4), the BC Employment Standards Act, the BC Human Rights Code, and the Workers Compensation Act. The Program Manager owns all HR decisions for their program; the Executive Director and HR are required for Step 4 discipline and termination.

3.1 Standard G.1 — Recruitment, Screening, and Personnel Files

MCFD Standard: G.1 — Thorough selection and screening procedures

Criminal Record Check: Vulnerable Sector clearance is an absolute non-negotiable requirement. No candidate works with youth in any capacity — supervised or unsupervised — until a clear CRC is received and documented. Renewed every 5 years. Any lapse: staff removed from youth-facing duties immediately.

Screening Process: Written application and résumé; personal interview (PM participates for all PC and senior direct-care positions); minimum 2 documented professional references reviewed before offer; educational and professional qualifications verified; Indigenous candidates supported with cultural liaisons on request

Youth Input: Children and youth are provided an opportunity to express opinions about the qualities they find helpful or unhelpful in staff. This input is documented and considered in personnel decisions. Documented at each ISP review.

Required Personnel File Contents: All 14 items required — verified by PC within 30 days of hire, PM signs off: (1) CRC Vulnerable Sector clearance; (2) personal and employment references; (3) relevant medical history; (4) application/résumé and interview record; (5) educational/professional qualifications verification; (6) signed offer letter and acceptance; (7) signed orientation acknowledgement; (8) all training certificates; (9) written performance evaluations; (10) documentation of any discipline; (11) dates of commencement and termination; (12) rehire recommendation; (13) exit interview record; (14) signed copies of all policies and updates

Job Descriptions: Current written job descriptions for every role; updated annually in July or upon any material role change; approved by PM and ED

 **Governing documents:** *HR Policy Binder s.3–4; PC Manual s.8.1–8.2; PM Manual s.8.1*

3.2 Standard G.2 — Staffing Ratios and Scheduling

MCFD Standard: G.2 — Sufficient staff to achieve program purpose

Minimum Ratios: Per CCALA licence conditions and Contracted Service Agreement; where these differ, the higher standard prevails. Never schedule below minimum. On-call supervisor available 24/7. Minimum 15-minute verbal handover between outgoing and incoming shift lead at every shift change.

Below-Minimum Protocol: Shift lead notifies PC immediately; PC notifies PM; PM takes all reasonable steps to arrange coverage within 2 hours; if minimum not restored, MCFD social worker notified and CI Report completed documenting all steps taken

Emergency Coverage Plan: Written plan maintained by PM; updated at minimum annually; includes current casual/relief staff list; PM reviews quarterly

Enhanced Staffing Triggers: Youth with active high-risk safety plan; two or more high-acuity youth simultaneously; overnight solo shift with youth with elopement or self-harm risk



Scheduling Requirements: Published minimum 2 weeks in advance; minimum 8-hour break between shifts; on-call schedule published weekly; fatigue management monitored by PM through monthly overtime review

 **Governing documents:** *HR Policy Binder s.2.4, s.6.4; PC Manual s.8.3; PM Manual s.8.2*

3.3 Standard G.3 — Mandatory Training Matrix

MCFD Standard: G.3 — Ongoing training to meet job requirements

Compliance Rule: Staff with lapsed mandatory training are removed from duties requiring that training immediately. No exceptions. PC updates training matrix monthly by the 5th; PM reviews by the 20th. HR issues monthly compliance report to PM and ED.

Training	Initial Deadline	Renewal	Platform / Provider
OHS Orientation + Program Manual	Day 1 — before first independent shift	Annual update; re-acknowledgement	In-person with PC
Youth Rights (CFCSA s.70)	Before first solo shift (within 7 days)	Every 2 years	BC Gov eLearning Hub (learning.gov.bc.ca)
Mandatory Reporting (CFCSA s.14)	Within 30 days	Every 2 years	mcf.gov.bc.ca/reporting
Privacy and FOIPPA	Within 30 days	Annual	oipc.bc.ca module
WHMIS 2015	Before first shift	Every 3 years	worksafebc.com
Abuse Recognition and Prevention	Within 30 days	Annual	Provider assigned by PM
Missing Child/Youth Protocol	Within 30 days	Annual (update when protocol revised)	mcf.gov.bc.ca; MCFD policy
Workplace Violence Prevention	Before first shift	Annual	worksafebc.com
First Aid / CPR Level C	Within 30 days; before first unsupervised shift	Per certification cycle	Red Cross, St. John, or equivalent
OFA Level 2 (designated FAA per shift)	Before first duty as designated FAA	Per OFA 2 cycle	WorkSafeBC-recognized provider
Non-Violent Crisis Intervention (NVCI)	Module 1 before solo shift; full cert within 90 days	Annual recertification	crisisprevention.com + in-person
Trauma-Informed Practice	Within 60 days	Every 2 years	BC Campus (bccampus.ca)
Indigenous Cultural Safety and Humility	Within 60 days	Annual	San'yas (sanyas.ca) or FNHA (fnha.ca)



Gender-Affirming Care — MANDATORY Standard G.3.2(b) amended April 2024	Within 60 days	Annual	Trans Care BC / PHSA (phsa.ca)
LGBTQ2S+ Inclusion	Within 60 days	Annual	PFLAG Canada (pflagcanada.ca)
Suicide and Self-Harm Response (ASIST or SafeTALK)	Within 60 days	ASIST: biennial; SafeTALK: annual	LivingWorks (livingworks.net)
Medication Administration (authorized staff only)	Before any medication duty	Annual competency check	BC medication admin curriculum + in-person
Working Alone Safety	Before first solo overnight shift	Annual	worksafebc.com
Driver Safety + annual driver's abstract	Before first driving duty	Annual abstract review	Employer orientation; ICBC

! Gender-Affirming Care training (Standard G.3.2(b), amended April 2024) is a MANDATORY MCFD Standard requirement — not optional. All staff must complete it within 60 days of hire and annually thereafter. Deliberate misgendering of any youth is a Code of Ethics violation subject to disciplinary action. This Standard also applies in supervision (Standard G.4.3(c)): every supervision session must address gender-affirming care practice.

Online Learning Platforms: WorkSafeBC (worksafebc.com); BC Gov eLearning Hub (learning.gov.bc.ca); MCFD (mcf.gov.bc.ca); San'yas (sanyas.ca); FNHA (fnha.ca); Trans Care BC/PHSA (phsa.ca); LivingWorks (livingworks.net); CPI (crisisprevention.com); BC Campus (bccampus.ca); PFLAG Canada (pflagcanada.ca); BC OIPC (oipc.bc.ca); ProQOL (proqol.org)

Governing documents: HR Policy Binder s.5; PC Manual s.8.4, Part 11; PM Manual s.8.3, Part 11; Orientation Manual Part 9

3.4 Standard G.4 — Supervision and Staff Support

MCFD Standard: G.4 — Staff receive support and supervision needed to perform their job requirements

Supervision Frequency: All direct-care staff: minimum monthly. New staff (first 90 days): bi-weekly. Staff on PIPs: bi-weekly. PM receives monthly supervision from ED.

Mandatory Supervision Content per G.4: Responses to and methods of working with children and youth; providing gender-affirming care to children and youth (G.4.3(c) — MANDATORY since April 2024); promoting safety and wellbeing; degree of personal involvement; individual feelings and concerns relating to job requirements; staff development and training

Communication Mechanisms: Individual supervision; team meetings (minimum monthly); shift handover meetings (minimum 15-minute overlap); shift logs and daily communications; all-staff meetings (PM leads, minimum quarterly)

Probationary Reviews: 30-day check-in (informal); 90-day formal review (written, both parties sign); 180-day end-of-probation review (formal confirmation of employment decision). All documented in personnel file.



Performance Evaluations: Annual written performance evaluations for all staff; performance rating scale documented; recognition programs

EFAP: Employee and Family Assistance Program available to all staff from Day 1; number posted in every staff area; use is confidential; actively promoted in supervision and following any Critical Incident

Secondary Traumatic Stress: Recognized occupational health risk in residential care; addressed in every supervision session; debriefs offered within 48 hours of every significant CI; scheduling decisions account for emotional load; ProQOL self-assessment (proqol.org) recommended annually

3.5 Performance Management and Progressive Discipline

Philosophy: Progressive discipline is intended to correct performance or conduct issues through a fair, consistent, and escalating process, giving employees a genuine opportunity to improve

Standard Steps: Step 1 (Verbal Warning): documented conversation; Step 2 (Written Warning): signed document in personnel file; Step 3 (Final Warning / PIP): signed PIP with defined timeline and consequences; Step 4 (Suspension or Termination): requires ED + HR; formal documentation

Summary Dismissal (immediate): Any act or suspected act of abuse, neglect, or exploitation of a child or youth; use of any prohibited practice (seclusion, corporal punishment, prone restraint); falsification of records; CRC-triggering disclosure. PM implements immediate suspension with pay pending investigation; ED and HR notified immediately; PM does not terminate unilaterally.

Grievance Procedure: Step 1: Written grievance to direct supervisor (within 20 business days); Step 2: Supervisor responds in writing (10 business days); Step 3: Escalation to PM (10 business days to respond); Step 4: Escalation to ED (final internal level). MCFD social worker grievances follow Contracted Service Agreement process.

Benefits and Compensation: Wage grid reviewed annually against sector benchmarks; shift premiums for evening, night, weekend; statutory holiday pay per ESA; extended health and dental; vacation entitlement per ESA and HR Policy; paid sick leave; ESA-protected statutory leaves; overtime per ESA with pre-approval requirement

 **Governing documents:** *HR Policy Binder s.7, s.13–17; PC Manual s.8.5; PM Manual s.8.4–8.5*



DIVISION 4 — FINANCE AND ADMINISTRATION

MCFD Standard H.1 · Finance and Administration Manual · GAAP

The Finance and Administration division governs all financial management, administrative systems, and organizational infrastructure of Evergreen Community Care. It is directly governed by MCFD Standard H.1, the Finance and Administration Manual, and applicable provincial and federal financial legislation. The Executive Director holds ultimate financial accountability; the Program Manager has delegated signing authority within defined limits.

4.1 MCFD Financial Management Requirements — Standard H.1

MCFD Standard: H.1 — Administration and management in an ethical and accountable manner

Accounting System: Maintained consistent with generally accepted accounting principles (GAAP)

Annual Budget: Produced annually; reviewed and approved by the Board before the fiscal year begins

Monthly Financial Statements: Accurately record all income and expenditures; reviewed by PM within 5 business days; PM signs off on any variance exceeding 10% of budget line

Annual Financial Report: Produced as required by MCFD; submitted by ED within contracted timelines

Financial Records: Stored in retrievable form; retained minimum 7 years (financial records); 10 years (medication records); accessible upon MCFD request

 **Governing documents:** *Finance and Administration Manual (ECC-FA-2026-001); PM Manual s.9.1*

4.2 Signing Authority and Delegated Financial Authority

Role	Signing Authority	Cannot Authorize
Youth Care Worker	Petty cash up to facility limit (per PM approval)	Anything beyond petty cash
Program Coordinator	Routine program purchases per Finance Manual delegated limit	Employment offers; contracts; capital expenditures
Program Manager	Program expenditures up to delegated limit; payroll exception authorization within policy	Employment offers (ED/HR); contractor agreements; capital expenditures above limit
Executive Director	All organizational commitments up to Board-approved limit; employment offers; contractor agreements; capital expenditures	Commitments exceeding Board-approved signing authority without Board resolution
Board of Directors	Organizational commitments exceeding ED authority; major capital; organizational borrowing	N/A — highest authority

Two-Signature Principle: Where possible, two authorized signatories required for payments above the single-signatory threshold; prevents internal fraud; both signatories independent



Conflict of Interest: Any staff member with a personal financial interest in a purchasing or contract decision must declare it immediately and recuse from the decision

 **Governing documents:** *Finance and Administration Manual s.4; HR Policy Binder s.3C*

4.3 Budget Development and Monitoring

Annual Cycle: September — PM prepares program-level budget submission; October/November — ED consolidates organizational budget; December — Board approves; January 1 — new fiscal year begins

Budget Structure: Revenue (MCFD per-diem, MCFD block, CLBC, other grants); Direct program costs (staffing, food, transportation, programming, client-related); Facility costs (rent, utilities, maintenance, insurance); Administrative overhead (HR, finance, IT, legal); Capital reserve

Monthly Monitoring: PM reviews program-level financial statements monthly; any variance >10% explained in writing; significant adverse variances escalated to ED within 5 business days

Contingency Reserve: Maintained per Finance Manual; approved by Board; accessed only with ED authorization for unbudgeted essential expenditures

 **Governing documents:** *Finance and Administration Manual s.5–6*

4.4 MCFD and CLBC Contract Revenue Management

Revenue Sources: MCFD per-diem funding (per occupied bed per day); MCFD block funding (where applicable); CLBC community living supports (where applicable); grant funding; other contracted services

Contract Financial Obligations: All contracted reporting deadlines met; financial data provided to MCFD accurately and on time; per-diem rates applied correctly; unearned revenue properly deferred; deferred funding recognized only when earned

Occupancy Tracking: PM tracks occupancy daily; reports to ED monthly; any pattern of under-occupancy (below 85%) that could trigger contract review discussed with ED and MCFD Contract Manager proactively

Contract Renewal: PM initiates financial analysis for contract renewal 6 months before expiry; PM and ED negotiate with MCFD Contract Manager; Board approves any material change to financial terms

 **Governing documents:** *Finance and Administration Manual s.7; PM Manual App. G*

4.5 Payroll, Purchasing, and Petty Cash

Payroll: Bi-weekly payroll by direct deposit; processed by Finance; PM approves all timesheets before submission; shift premiums, overtime, and on-call pay applied per HR Policy; PM reviews payroll report monthly

Purchasing: All purchases within delegated limits and consistent with the annual budget; receipts required for all purchases and submitted within 5 business days; no verbal commitments for goods or services above petty cash without written authorization

Petty Cash: Maintained at each facility per Finance Manual approved limit; reconciled monthly by PC; reviewed by PM; replenished through proper authorization; any discrepancy investigated immediately

Youth Allowances and Trust Funds: Youth allowances paid consistently on scheduled day — never withheld as a consequence (CFCSA s.70 violation); trust fund transactions fully documented; financial log submitted to PM monthly

 **Governing documents:** *Finance and Administration Manual s.8–10*



4.6 Insurance and Asset Management

Required Insurance: Commercial general liability; professional liability (errors and omissions); Directors and Officers (D&O); vehicle insurance per ICBC for all program vehicles; property/contents; volunteer liability. All policies reviewed annually by PM; confirmed adequate by ED; reported to Board.

MCFD Requirement: Standard H.1 — property, vehicle, and liability insurance adequate and appropriate for program operation; must include casual support staff, student placements, and volunteers

Annual Review: PM confirms all insurance policies annually in July; any gap or lapse reported to ED immediately; Board receives annual insurance summary

Asset Management: All significant organizational assets inventoried; disposed of only with appropriate authorization; vehicle maintenance per manufacturer schedule; defects reported immediately

 **Governing documents:** *Finance and Administration Manual s.11; RC Program Manual v4 s.10.1*



DIVISION 5 — QUALITY ASSURANCE

MCFD Standard H.2 · QA Program Binder · Continuous Improvement

The Quality Assurance division ensures Evergreen Community Care continuously examines whether children and youth in its care are safer, better supported, and more connected than they were before. QA is not an audit function — it is how the organization asks the honest questions that drive genuine improvement. It is governed by MCFD Standard H.2 and the QA Program Binder (ECC-QA-2026-001).

The QA Framework: Four Questions	Primary Tools
Are we safe? — Is every child protected from harm, every shift, every facility?	Facility Inspections (Tool B); Incident Trend Analysis (Tool C); Restraint Monitoring
Are we compliant? — Do our practices meet or exceed MCFD Standards A–H and licensing requirements?	MCFD Self-Assessment (Tool A); ISR File Audit (Tool D); Training Compliance Matrix
Are we improving? — Are outcomes getting better over time, not just staying the same?	KPI Dashboard; Annual Program Review; Corrective Action Log
Are we listening? — What are the children, youth, families, and staff telling us?	Youth and Family Feedback Survey (Tool F); Staff Survey (Tool G); Complaints Analysis

5.1 QA Roles and Responsibilities

Role	QA Responsibilities
Youth Care Worker / Shift Lead	Complete shift logs, MARs, and incident reports accurately and on time. Report hazards and near-misses. Participate in post-incident debriefs. Complete required training. Respond honestly in staff satisfaction surveys.
Program Coordinator	Conduct monthly facility safety inspections (Tool B). Monitor documentation quality. Track and report training compliance. Collect and submit youth/family feedback. Escalate QA concerns to PM. Implement assigned corrective actions.
Program Manager	Complete quarterly CI trend analyses (Tool C). Conduct bi-annual ISR file audits (Tool D). Lead annual MCFD Standards Self-Assessment (Tool A). Manage KPI tracking. Lead post-CI reviews. Present QA findings to ED quarterly. Lead the annual program review.



Executive Director	Approve QA Program and annual updates. Review quarterly KPI dashboard. Ensure annual program review is completed and submitted to MCFD. Hold PM accountable for QA activity completion. Approve corrective action plans for systemic issues.
Board of Directors	Receive annual program review and QA summary. Hold ED accountable for program quality and safety outcomes. QA is a standing Board agenda item minimum once per year.
JHSC	Conduct monthly OHS inspections. Review all WorkSafeBC incidents. Identify and recommend corrective actions for OHS-related QA findings. Contribute to annual program review where OHS data is relevant.

5.2 QA Calendar — All Activities by Frequency

Frequency	QA Activity	Responsible
Every shift	Shift log completed and signed; MAR current; medication count reconciled; safety walkthrough documented	Shift lead / YCW
Monthly — by 5th	Training Compliance Matrix updated (Appendix E of PC Manual)	Program Coordinator
Monthly — first week	Facility Safety Inspection (QA Tool B / PC Manual App. C)	Program Coordinator
Monthly — by 10th	Monthly QA Review Tool submitted to PM (PC Manual App. D)	Program Coordinator
Monthly — by 20th	PM reviews all monthly QA reports and CI logs; updates corrective action log	Program Manager
Quarterly	Critical Incident Trend Analysis (Tool C / PM Manual App. C)	Program Manager
Quarterly	KPI dashboard reviewed at ED-PM operations meeting	Executive Director + PM
Quarterly	All-staff meeting (PM leads)	Program Manager
Bi-annual (January and July)	ISR File Audit (Tool D / PM Manual App. D) — minimum 25% of active files	Program Manager
Bi-annual (January and July)	Youth and Family Feedback Survey (Tool F)	Program Coordinator
Annual — July	MCFD Standards Self-Assessment A–H (Tool A / PM Manual App. E)	Program Manager + ED



Annual — July	Annual Program Review per MCFD Standard H.2	Executive Director
Annual — July	Staff Satisfaction and Wellbeing Survey (Tool G)	Human Resources
Annual — July	QA Binder review and update	Executive Director
Annual — July	All policy manuals reviewed and updated; staff sign acknowledgements within 30 days	ED + PM + PC

5.3 Key Performance Indicators

KPI	Target	Measurement Frequency
Critical incidents requiring MCFD notification	Decreasing trend year-over-year	Monthly / Quarterly
CI reports submitted to MCFD within required timelines	100%	Monthly
Physical restraint incidents	Zero preferred; decreasing trend; zero prohibited techniques	Monthly
Incidents involving prohibited practices	Zero — zero tolerance	Monthly
Missing child/youth incidents	Zero unresolved; decreasing trend	Monthly
Medication errors	Zero; decreasing trend	Monthly
Fire drills completed on schedule	100%	Quarterly
WorkSafeBC reportable injuries to staff	Decreasing trend	Monthly
Staff mandatory training compliance	100%	Monthly
ISP reviews completed on schedule (90-day cycle)	100%	Bi-annual file audit
Cultural Plans for	100%	Bi-annual file audit



Indigenous youth — within 30 days		
MCFD licensing — critical deficiency findings	Zero	Annual inspection
CRC expiries — active staff	Zero	Monthly
Youth satisfaction score (Tool F)	Positive / improving trend	Annual survey
Staff retention rate	85%+	Annual HR review
Corrective actions completed by target date	90%+	Quarterly log review
Complaints resolved within 10 business days	100%	Quarterly log review

5.4 Annual Program Review — MCFD Standard H.2

MCFD Standard: H.2 — Program review and evaluation; promotes quality of service

Required Coverage: Need for and relevance of services; attainment of goals related to each child's plan of care; attainment of overall program goals and objectives; human and material resource management; current relevance of policy and procedure

Who Participates: Executive Director (leads); Program Manager (provides data); Program Coordinators (contribute); Board of Directors (receives report); MCFD (receives summary report per Service Agreement); staff survey data included; youth and family survey data included

Corrective Action Process: Any gap identified in QA activities triggers a corrective action: documented in the Corrective Action Log with finding, root cause, action required, named owner, target date, and follow-up date. PM tracks weekly; reports overdue items to ED monthly; ED escalates systemic issues to Board.

 **Governing documents:** QA Program Binder (ECC-QA-2026-001); PM Manual s.9.2, App. C–E



DIVISION 6 — OHS AND WORKPLACE SAFETY

Workers Compensation Act · OHS Regulation · WorkSafeBC · JHSC

The OHS and Workplace Safety division governs every aspect of health and safety for Evergreen Community Care workers. It is governed by the Workers Compensation Act, the BC OHS Regulation, and WorkSafeBC policies. Every person at every level of the organization has OHS rights and responsibilities. The Program Manager is the primary management accountable at the facility level under WCA s.115.

6.1 OHS Roles and Responsibilities (WCA s.115)

Role	OHS Accountability
Board of Directors	Approves OHS Program, annual OHS budget; receives annual OHS performance report; ensures adequate resources for OHS compliance
Executive Director	Ultimate operational OHS accountability; reviews all serious incidents; ensures corrective actions completed systemically; signs annual OHS report
Program Manager	Statutory management accountability at facility level (WCA s.115); reviews all incident investigations; responds to JHSC recommendations in writing within 21 days; conducts annual violence risk assessment; responds to WorkSafeBC orders
Health and Safety Coordinator	Day-to-day OHS administration; maintains all OHS records; acts as primary WorkSafeBC liaison; supports and coordinates the JHSC; tracks OHS training compliance; maintains OHS bulletin boards
Program Coordinator	Implements OHS Program at worksite level; conducts monthly inspections; investigates incidents; ensures new worker OHS orientation; maintains working-alone check-in log; maintains WHMIS/SDS binder
All Workers	Work safely; use PPE; report all hazards and near-misses immediately; refuse unsafe work without fear of retaliation; cooperate in investigations; protect coworkers and youth in care
JHSC	Monthly meetings; workplace inspections; incident investigation participation; work refusal response; annual OHS program review; written recommendations to management within 21-day response window

6.2 Joint Health and Safety Committee (JHSC) — WCA s.125

Legal Requirement: Required for workplaces regularly employing 20+ workers. Facilities under 20 workers designate a Worker Health and Safety Representative integrated into the organization-wide JHSC.



Composition: Minimum 4 members; at least half must be worker (non-management) representatives; worker representatives selected by workers; two co-chairs (one worker, one management); 2-year staggered terms; membership posted on OHS bulletin board at every facility

Statutory Functions: Identify hazards and recommend controls; participate in inspections and investigations; receive and investigate worker safety complaints (respond in writing within 21 days); participate in work refusals; review and recommend improvements to OHS Program; promote OHS education; advise on training requirements

Meeting Requirements: Monthly minimum; quorum = one worker + one management representative; minutes posted within 7 days and retained 2 years; annual summary to ED and Board; emergency meeting within 48 hours of any fatality, critical injury, or WorkSafeBC stop-work order

Management Response to JHSC: Management must respond in writing to every JHSC recommendation within 21 days. No exceptions. PM owns this obligation at the facility level; ED at the organizational level.

 **Governing documents:** *OHS Program Binder (ECC-OHS-POL-2026-002) s.3; PM Manual s.10*

6.3 Hazard Identification and Risk Assessment

Key Hazards — Residential Care: Workplace violence (highest risk — must be assessed annually); infection and bloodborne pathogens; medication handling errors; working alone (overnight and solo shifts); musculoskeletal injury (personal care, restraint); slips, trips, and falls; chemical hazards (WHMIS); vehicle/transport; secondary traumatic stress and compassion fatigue

Annual Violence Risk Assessment: Conducted by PM in consultation with JHSC annually (July); also following any serious violent incident. Covers: building layout; supervision gaps; youth population acuity; de-escalation supports; environmental design; staff training; post-incident supports.

Working Alone: Risk assessment required for every solo overnight shift (OHS Reg. s.4.21–4.24). Check-in at required intervals (typically every 2 hours). Escalation protocol: failed check-in → attempts to reach worker → 911 if unsuccessful. PM reviews working-alone check-in log monthly.

WHMIS: All hazardous products labeled; Safety Data Sheets (SDS) current and accessible for all chemicals in use; staff trained before exposure; SDS binder updated within 30 days of any product change; confirmed current at monthly inspection

 **Governing documents:** *OHS Program Binder s.4–6; PC Manual s.10*

6.4 WorkSafeBC Incident Reporting and Investigation

Incident Type	Required Action	Timeline / Authority
Serious incident: hospitalization, fracture, amputation, loss of consciousness, fatality	Immediate phone report to WorkSafeBC: 1-888-621-7233	Immediately by phone (WCA s.172); PM is accountable
Any work-related injury requiring medical attention beyond first aid	WorkSafeBC Form 7 (employer's report)	Within 3 business days of learning of injury (WCA s.150); PM confirms submission



All incidents and near-misses	Incident Investigation Report completed by PC; co-signed by PM	Investigation within 24 hours; report within 48 hours; PM co-signs all reports
Worker right-to-refuse (OHS Reg. s.3.12)	PM responds promptly; follow the WCA work refusal process; never retaliate against a refusing worker	Immediately on receipt; PM leads management response
WorkSafeBC inspection order or stop-work order	Comply with order; notify ED immediately; respond within required timeline	Per order timeline; PM leads; ED notified immediately

6.5 Residential Care Specific Safe Work Procedures

De-escalation Safety: NVCI training required for all direct-care staff who may need to use physical intervention; de-escalation is always the first response; never engage alone in any high-risk de-escalation situation; backup called before physical intervention; no prone restraint under any circumstances

Medication Handling: Controlled substances in double-locked storage; count reconciled at every shift change; discrepancy investigated same shift; expired medications returned through BC Medications Return program; PPE (gloves) for all medication administration

Infection Prevention and Control: Standard precautions for all direct care; PPE (gloves, masks, eye protection) accessible at every facility; blood and bodily fluid exposure response posted; hand hygiene stations at all care points; communicable disease protocol in place

Nighttime Monitoring: Working-alone protocol active for all overnight solo shifts; wellness checks per each youth's ISP (minimum hourly for all, every 30 minutes for youth with active safety plans); check-in log completed before end of shift

Return-to-Work: RTW plan initiated within 48 hours of any work-related injury; modified duty options identified by PM; regular supportive contact with worker on leave (minimum bi-weekly); no pressure or penalty for filing WorkSafeBC claim; PM coordinates with HR

 **Governing documents:** OHS Program Binder s.5–6, s.11; PM Manual s.10; PC Manual s.10



DIVISION 7 — EXTERNAL RELATIONS

MCFD · Licensing · Indigenous Community · RCY · WorkSafeBC · Legal

The External Relations division governs Evergreen Community Care's relationships with every external party — regulatory, contractual, community, and legal. The Executive Director is the organizational face for all external relationships with material implications. The Program Manager manages day-to-day MCFD and licensing relationships. No staff member speaks to media without ED authorization.

7.1 MCFD Contract Management

Primary Contact: Program Manager for day-to-day program-level relationship; Executive Director for contract negotiations, material issues, and formal complaints

Contract Tracking: PM maintains the MCFD Contract Tracking Log (PM Manual Appendix G) with all reporting deadlines, deliverables, and key dates; reviewed monthly; PM never misses a contracted deadline

Reporting Obligations: Monthly CI trend summary; training compliance data (if contracted); quarterly program update; annual program review submission; financial reporting per Service Agreement; youth outcome data; staff roster updates. All submitted on time, every time.

Proactive Communication: PM notifies MCFD Contract Manager proactively of any material service issue, capacity constraint, or compliance risk — before MCFD learns from another source. This is a non-negotiable professional standard.

Monitoring Visits: PM facilitates all MCFD monitoring visits promptly and professionally; provides access to all records, staff, and youth on request; never delays or obstructs; responds to monitoring findings within required timelines

Contract Renewal: PM initiates renewal process 6 months before expiry; ED leads negotiations; Board approves any material change to financial terms; licence renewal initiated 90 days before expiry with application submitted 60 days before expiry

Social Worker Relationship: PM maintains minimum monthly contact with each youth's MCFD social worker — not only when problems arise. MCFD social workers have unrestricted access to all youth, records, and facilities at all times.

 **Governing documents:** *PM Manual s.3.3, s.9.3, App. G; RC Program Manual v4 s.14*

7.2 Health Authority Licensing (CCALA)

Primary Contact: Program Manager for facility-level licensing relationship; Executive Director receives any licensing order, licence suspension, or stop-work order

Licence Conditions: Program operates within all licence conditions at all times — capacity, staffing model, physical standards, population served. Licensing Officer notified of any change before implementation.

Inspection Protocol: Facilitate all Licensing Officer inspections promptly and professionally; never obstruct or delay; provide all requested records immediately; PM present and engaged during all inspections

Deficiency Response: Written corrective action plan submitted within the timeline specified in the inspection report. PM leads; ED receives copy. Priority 1 deficiencies corrected before or simultaneously with notification.

Licence Renewal: PM maintains Licence Renewal Calendar; renewal application submitted minimum 60 days before expiry; ED countersigns



Escalation: Any licensing order, licence suspension, stop-work order, or formal compliance requirement — notify ED immediately on receipt; ED leads the organizational response

 **Governing documents:** *PM Manual s.2.3; RC Program Manual v4 s.14*

7.3 Indigenous Community, Nation, and DAA Relations

Legislative Foundation: Declaration on the Rights of Indigenous Peoples Act (DRIPA, SBC 2019, c.44); CFCSA s.2, 70, 71; TRC Calls to Action (particularly Call to Action 65 on Indigenous child welfare); UNDRIP Articles 7, 8, 14, and 21

Placement Priority: When residential care is required for an Indigenous child, family-based placement preference applies and must be genuinely pursued before group care: (1) extended family; (2) family-nominated persons; (3) Nation-approved family; (4) another Indigenous family; (5) non-Indigenous family committed to cultural support; (6) group care only when all family-based options are genuinely unavailable

Nation Notification: Nation is notified of any placement of a member child; DAA (where applicable) is engaged from Day 1; DAA involvement is facilitated in ISP and Cultural Plan development

Cultural Plan: Developed within 30 days of admission for every Indigenous child or youth; genuine collaboration with youth, family, and Nation; updated at every ISP review; documents: Nation and community affiliation, Elders and cultural mentors, specific activities and ceremonies, traditional foods and medicines, language learning, DAA involvement and responsibilities

Staff Obligations: All staff complete Indigenous Cultural Safety and Humility training within 60 days of hire (annual renewal). Smudging, ceremony, drumming, traditional foods, and cultural dress accommodated without restriction or judgment. Any racially discriminatory comment or action treated as a Critical Incident.

Ongoing Relationship: PM maintains active relationship with Nation and DAA representatives beyond specific youth placements; participates in community events and consultation processes as appropriate; advances DRIPA implementation in practice

 **Governing documents:** *RC Program Manual v4 s.31; PC Manual s.8; PM Manual s.3*

7.4 Community and Service Provider Partnerships

School Districts: Enrollment within 5 school days of admission; ongoing school liaison relationship maintained by PC; PM attends all IEP reviews and case conferences; school suspensions reported as Category 1 CIs

Health Authorities: Youth established with GP or NP within 30 days; specialist referrals tracked and attended; mental health services connected within 30 days of identification; HealthLink BC (8-1-1) used for nurse-line consultations

Mental Health and Substance Use Services: Active referral relationships with regional mental health and substance use services; Foundry centres connected for youth approaching 19; peer support networks developed as part of ISP planning

Cultural and Community Organizations: Friendship Centres, Indigenous cultural organizations, recreation programs, faith communities, and cultural groups connected for each youth per their ISP and Cultural Plan

Employment and Education Programs: Connections to employment programs, post-secondary institutions, and Agreements with Young Adults (AWA) for youth aging out of care; transition planning begins at 16 for youth approaching 19

 **Governing documents:** *RC Program Manual v4 s.22–25; PC Manual s.6*



7.5 RCY, Ombudsperson, and External Oversight Bodies

Representative for Children and Youth (RCY): All youth have unrestricted access to the RCY at any time, without asking permission, in private: 1-800-476-3933. Staff actively facilitate this access. Evergreen Community Care cooperates fully with all RCY investigations and reviews. RCY cannot be prevented from contacting any youth.

BC Ombudsperson: All youth have unrestricted access: 1-800-567-3247. All complaints to the Ombudsperson are cooperated with fully and promptly.

OIPC: All privacy complaints and Information Commissioner investigations are cooperated with fully; ED leads organizational response; records are preserved and not altered; requested documents provided within legislative timelines

WorkSafeBC: All WorkSafeBC inspections facilitated immediately and cooperated with fully; no alteration of records or workplace conditions before a WorkSafeBC inspector arrives; all orders complied with within specified timelines; Health and Safety Coordinator is primary WorkSafeBC liaison

 **Governing documents:** *RC Program Manual v4 s.4; OHS Program Binder s.2; PM Manual s.3*

7.6 Media, Legal, and Public Communications

Media Authority: The Executive Director is the ONLY authorized spokesperson for Evergreen Community Care with media. No other staff member at any level speaks to media about any program, any youth, or any incident — even to confirm or deny basic facts. All media inquiries are immediately redirected to the ED by name and contact.

Social Media: Staff do not discuss any youth, any placement, or any organizational matter on any personal or organizational social media platform. Photographing any youth is prohibited without written MCFD authorization. This policy applies to all staff, students, and volunteers.

Legal Counsel: Organizational legal counsel is engaged by the ED for: any legal proceeding; any regulatory investigation with potential organizational liability; any employment law matter beyond internal progressive discipline; any insurance claim. PM does not engage legal counsel without ED authorization.

Privacy of Youth: Never confirm or deny to any unauthorized person that any specific youth is in government care. Never share youth name, location, physical description, family relationships, or placement history with anyone not authorized. This applies to police, family members, community members, and media.

 **Governing documents:** *HR Policy Binder s.11.9; RC Program Manual v4 s.17.3*



CROSS-DIVISION INTERDEPENDENCY MAP

The following events trigger responses across multiple divisions simultaneously. Every person in the organization must understand which divisions activate on the trigger events most relevant to their role.

Triggering Event	Divisions Activated	Primary Governing Document	Timeline
New youth admitted to program	Program Operations (admission, ISP, rights); HR (staffing ratios, CRC confirmation); Finance (per-diem tracking); QA (admission checklist, ISP compliance tracking)	RC Program Manual v4 s.16; PC Manual App. B	ISP preliminary: 48 hrs; full ISP: 14 days; Cultural Plan: 30 days
Category 1 Critical Incident occurs	Program Operations (immediate response, CI report); OHS (if worker injury — WorkSafeBC notification); QA (post-CI review, corrective action); External (MCFD notification within 2 hours); Governance (ED notification same day)	RC Program Manual v4 s.26; PM Manual s.4; OHS Binder s.11	MCFD: within 2 hrs; CI Report: 24 hrs; Post-CI review: 5 business days
New staff member hired	HR (CRC, file, orientation); Program Operations (schedule, supervision assignment); OHS (OHS orientation before first shift); Finance (payroll setup, expense authorization)	HR Policy Binder s.3–4; PC Manual App. B; OHS Binder s.3.22	CRC: before Day 1; OHS orientation: Day 1; training compliance: within 30–90 days per matrix
Mandatory training lapses for any staff member	HR (training compliance, possible discipline); Program Operations (duty removal immediately); QA (KPI impact, training matrix update)	PC Manual App. E; HR Policy Binder s.5	Duty removal: immediately on identification; compliance restoration: before return to duty
Licensing inspection occurs	Governance (ED notification on receipt of any order); Program Operations (inspection facilitation, deficiency response); QA (self-assessment triggered); External (Licensing Officer relationship)	PM Manual s.2.3; QA Binder Tool A; CCALA	Facilitate immediately; corrective action plan: per inspection report timeline
Indigenous youth admitted	Program Operations (Cultural Plan within 30 days, Nation notification Day 1); External (DAA engagement, Nation notification); QA (Cultural Plan	RC Program Manual v4 s.31; CFCSA s.71; DRIPA	Nation notification: Day 1; Cultural Plan meeting: within 30 days; Cultural Plan complete: 30 days



	tracking on admission checklist)		
Worker injury occurs	OHS (incident investigation, WorkSafeBC reporting); HR (RTW planning, WSBC claim support); Finance (RTW modified duty budget); QA (incident log, corrective action)	OHS Binder s.11; WCA s.150, 172; PM Manual s.10.2	Serious: phone WSBC immediately; Form 7: 3 business days; RTW plan: 48 hours
Youth rights violation alleged (by staff or about staff)	Program Operations (immediate response, CI report, suspension if staff conduct); HR (progressive discipline / summary dismissal); External (MCFD notification same day); Governance (ED notification same day)	RC Program Manual v4 s.4; HR Policy Binder s.13.5; CFCSA s.70	Suspension: immediate; MCFD: same day; Investigation: within 24 hours
Annual review cycle (July)	All seven divisions participate: Governance (Board review); Program Operations (Standards A–F self-assessment); HR (G.1–G.4 review); Finance (budget cycle); QA (full program review, all tools); OHS (OHS performance review); External (contract and licence renewal tracking)	QA Program Binder s.5; PM Manual s.12.5; RC Program Manual v4 s.38.3	All activities: complete by July 31; Annual Program Review: submitted to MCFD per Service Agreement

Version History

Version	Date	Summary
1.0	July 1, 2026	Initial Organizational Divisions document; all seven divisions defined; all areas, roles, governing standards, and documents aligned to MCFD SHSS Standards A–H (revised April 2026); mandatory gender-affirming care training Standard G.3.2(b) and supervision requirement G.4.3(c) (April 2024 amendments) included; RCR April 2026 hot water temperature amendment (49°C) included; all training platforms and cross-division interdependency map included

Signatory	Signature / Date
Executive Director	Signature: _____ Date: _____
Board Chair	Signature: _____ Date: _____